

NO PRIOR AUTHORIZATION

Hormone Therapy *Playbook*

BHRT FOR AESTHETIC PRACTICES

7 Modules · 100+ Pages

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The Hormone Therapy Playbook

INCORPORATING BHRT · PATIENT SCREENING · PROTOCOLS · VENDORS · CONSENT FORMS · FAQs

1

Hormone Therapy — Overview & Scope of Practice

What BHRT is, who can prescribe it, and why it belongs in your aesthetic practice

READ THIS FIRST — SCOPE OF PRACTICE IS EVERYTHING

Hormone therapy prescribing scope varies significantly by state and by license type. NPs and PAs can prescribe in most states independently or with a collaborative agreement. MDs and DOs prescribe independently. Estheticians and non-prescribers cannot prescribe hormones — ever. Before building a hormone program, verify your state's prescribing authority and collaborative agreement requirements. This playbook is an operational guide — not a substitute for legal and regulatory review of your specific license and state.

What BHRT is — and why patients are seeking it

BIOIDENTICAL HORMONE REPLACEMENT THERAPY (BHRT)

BHRT uses hormones that are chemically identical to those naturally produced by the human body — estradiol, progesterone, testosterone, DHEA, and others. Unlike synthetic hormones, bioidentical hormones are derived from plant sources and compounded or manufactured to match the body's natural molecular structure. Patients seek BHRT for: perimenopause and menopause symptoms, fatigue, weight gain, low libido, brain fog, mood instability, sleep disruption, and loss of muscle mass.

Why this belongs in your aesthetic practice

THE NATURAL CONNECTION TO AESTHETICS

- Hormonal decline directly causes skin thinning, collagen loss, hair thinning, and weight redistribution — the same concerns aesthetic patients present with
- A patient on optimized hormones responds better to aesthetic treatments — better skin quality, better healing, better filler retention
- Hormone patients are high-retention — monthly visits, quarterly labs, ongoing management
- Average hormone patient LTV: \$2,400–\$4,800/year before any aesthetic services

- Natural cross-sell: hormone patients are already invested in their wellness and open to aesthetic services

\$4.2B

US hormone therapy market — growing 7% annually

6,000+

Women enter menopause daily in the US

\$200–\$400

Average monthly revenue per hormone patient

How to incorporate — the three models

1

In-house prescribing (NP/PA/MD)

You prescribe, manage, and monitor. Highest revenue. Highest responsibility. Requires prescribing authority, collaborative agreement if required, lab ordering capability, and a compounding pharmacy relationship. Full clinical accountability stays with your practice.

2

Collaborative model — partner with a hormone specialist

You identify and refer, the specialist prescribes, you administer (if within scope) and co-manage the aesthetic component. Revenue share or referral arrangement. Lower liability, lower revenue. Good starting point.

3

Telehealth platform integration

Partner with a telehealth hormone platform (Biote, Evexias, Winona, Midi Health). They handle prescribing. You administer pellets or injections if applicable and earn a facility fee. Simplest entry point — least infrastructure required.

2

Patient Screening & Consultation

Who is a candidate, who isn't, and how to open the conversation

OPENING CONSULTATION SCRIPT

"Tell me how you've been feeling — not just physically, but overall. Energy, mood, sleep, your relationship with your body. I want the full picture before we talk about any options."

Hormone consultations are emotional. Women have often been dismissed by other providers for years before arriving at your door. Lead with listening. The clinical assessment comes after they feel heard.

Ideal hormone therapy candidates

SYMPTOMS THAT INDICATE HORMONE EVALUATION

- Perimenopause or menopause — hot flashes, night sweats, irregular cycles, vaginal dryness
- Fatigue that doesn't resolve with sleep — especially mid-afternoon crashes
- Low libido or sexual dysfunction — in both women and men
- Brain fog, difficulty concentrating, memory concerns
- Mood instability — anxiety, irritability, depression not fully explained by other causes
- Unexplained weight gain — especially abdominal, despite diet and exercise

- Loss of muscle mass or difficulty building muscle despite training
- Hair thinning or loss beyond normal shedding
- Sleep disruption — difficulty falling or staying asleep

Contraindications — absolute and relative

ABSOLUTE CONTRAINDICATIONS — DO NOT PRESCRIBE ESTROGEN

- Personal history of estrogen receptor-positive breast cancer
- Undiagnosed abnormal vaginal bleeding
- Active or recent DVT, PE, or arterial thromboembolic disease
- Active liver disease or liver failure
- Known or suspected pregnancy
- Endometrial cancer (estrogen without progestogen)

RELATIVE CONTRAINDICATIONS — DISCUSS RISK/BENEFIT CAREFULLY AND DOCUMENT

History of migraines with aura, hypertriglyceridemia, active gallbladder disease, uterine fibroids (size-dependent), history of clotting disorders, strong family history of hormone-sensitive cancers. These require individualized risk-benefit discussion, shared decision-making, and thorough documentation.

Baseline labs — required before prescribing

FEMALE HORMONE PANEL — MINIMUM BASELINE

- Estradiol (E2) — baseline level, guides dosing
- Progesterone — especially important in perimenopausal patients
- Total and free testosterone — often deficient in women; impacts libido, energy, muscle
- FSH and LH — confirm menopausal status
- DHEA-S — adrenal hormone, often declines with age
- SHBG (sex hormone binding globulin) — affects free hormone availability
- TSH, free T3, free T4 — thyroid function; overlapping symptoms with hormone imbalance
- Complete metabolic panel — liver function, kidney function, glucose
- CBC — baseline blood counts
- Lipid panel — cardiovascular baseline
- Fasting insulin — metabolic health indicator
- Vitamin D, B12 — commonly deficient; affect similar symptoms

MALE HORMONE PANEL — TESTOSTERONE OPTIMIZATION

- Total testosterone — baseline; low T defined as <300 ng/dL by most guidelines
- Free testosterone — more clinically relevant than total in symptomatic patients
- Estradiol (sensitive assay) — men convert testosterone to estrogen; monitor to prevent excess
- LH and FSH — differentiate primary vs. secondary hypogonadism
- PSA — baseline and monitoring for men on testosterone therapy
- Hematocrit/CBC — testosterone increases RBC production; monitor for polycythemia
- CMP, lipids, thyroid — full metabolic baseline

3

Hormones, Delivery Methods & Dosing Protocols

The major hormones, how they're delivered, and general dosing frameworks

IMPORTANT — DOSING IS INDIVIDUALIZED

Hormone dosing is not a one-size-fits-all protocol. Every dose should be based on the patient's labs, symptoms, body weight, age, and clinical response. The ranges below are general starting frameworks — not fixed protocols. Always dose to symptom resolution and optimal lab values, not to a number on a chart.

Estrogen — women's foundational hormone

DELIVERY METHOD	STARTING RANGE	NOTES
Transdermal patch Avoids first-pass liver metabolism — preferred for clot risk	0.025–0.05mg/day	Change twice weekly or weekly depending on product. Lower clot risk than oral.
Transdermal gel/cream Good absorption, flexible dosing	0.5–1.5g/day	Apply to inner arm or thigh. Avoid breast tissue. Wash hands after application.
Vaginal estrogen Local effect — minimal systemic absorption	Per product guidelines	Does not require progestogen in most cases. Excellent for GSM symptoms.
Pellet therapy Subcutaneous — sustained release 3–6 months	6–12.5mg (varies by weight/labs)	Inserted subcutaneously — typically gluteal area. Requires training and insertion certification.
Oral estradiol Convenient — higher clot risk	0.5–2mg/day	Higher VTE risk than transdermal. Avoid in patients with clotting history.

Progesterone — essential with uterus intact

CLINICAL RULE — ALWAYS PRESCRIBE PROGESTOGEN WITH SYSTEMIC ESTROGEN IF UTERUS IS PRESENT

- Micronized progesterone (Prometrium) — bioidentical, preferred over synthetic progestins
- Typical dose: 100–200mg nightly (cyclic or continuous depending on menopausal status)

- Compounded progesterone cream: commonly used but limited systemic absorption — not adequate for endometrial protection
- Benefits beyond uterine protection: improved sleep, mood stabilization, anxiety reduction
- Women without a uterus: progestogen not required but may be used for symptomatic benefit

Testosterone — for women AND men

PATIENT	DELIVERY	STARTING RANGE	MONITOR
Women	Compounded cream/gel (off-label)	0.5–2mg/day topically	Total T, free T, SHBG, hematocrit at 6–8 weeks
Women	Pellet (off-label)	50–100mg per insertion	Labs at 4–6 weeks post-insertion
Men — TRT	Testosterone cypionate IM	50–100mg weekly or 100–200mg biweekly	Total T, free T, E2, hematocrit, PSA at 6–8 weeks
Men — TRT	Topical gel (AndroGel, compounded)	Per product; 50mg/day starting	Same as above. Transfer risk to partners/children.
Men — TRT	Pellet	900–1200mg per insertion (weight-based)	Labs at 4–6 weeks. PSA monitoring essential.

DHEA — the adrenal androgen precursor

CLINICAL OVERVIEW

- Available OTC as a supplement — also compounded for specific dosing
- Typical dose: 25–50mg daily oral; 10–25mg for women (lower doses)
- Prasterone (Intrarosa) — FDA-approved vaginal DHEA for dyspareunia in menopause
- Baseline DHEA-S required before supplementing — check adrenal status
- Caution: can convert to androgens — monitor for androgenic side effects in women

Follow-up and monitoring schedule

- 6–8 weeks post-initiation**
Repeat hormone panel. Assess symptom response. Adjust dosing based on labs AND symptoms — not labs alone. Some patients need levels at high-normal range for symptom resolution.
- 3 months**
Full repeat panel. Assess tolerability. Check for androgenic side effects if testosterone prescribed. PSA in men. Hematocrit for testosterone patients.
- Every 6 months (stable patients)**
Maintenance labs. Reassess need for continued therapy. Annual mammogram reminder for estrogen patients. Continued PSA monitoring for men.

Vendors, Compounding Pharmacies & Lab Partners

Who to work with, what to look for, and how to vet your partners

VENDOR VETTING IS A PATIENT SAFETY RESPONSIBILITY

Compounded hormones are not FDA-approved finished drug products. Quality, potency, and sterility vary between compounding pharmacies. You are responsible for the quality of what you prescribe. Always use PCAB-accredited compounding pharmacies and verify 503A or 503B status depending on your practice model.

Compounding pharmacy partners — what to look for

Belmar Pharmacy

COMPOUNDING · PCAB ACCREDITED

Specializes in hormone replacement therapy. Offers broad BHRT compounding including pellets, creams, troches, and injectables. Strong prescriber support and educational resources. PCAB accredited — high quality standard.

→ Good for practices starting a full BHRT program

Empower Pharmacy

COMPOUNDING · 503B OUTSOURCING

Large 503B outsourcing facility. FDA-registered. Sterile and non-sterile compounding. Competitive pricing. Wide formulary including testosterone cypionate, estradiol, progesterone. Ships nationwide. Good for high-volume practices.

→ Strong option for injectable testosterone programs

Women's International Pharmacy

COMPOUNDING · WOMEN-FOCUSED

Specializes exclusively in women's health compounding. BHRT creams, troches, capsules, vaginal preparations. Excellent patient education materials. Prescriber support line available.

→ Best for women-only hormone programs

Biote Medical

PELLET PROGRAM · TRAINING INCLUDED

Turnkey pellet therapy program. Includes prescriber training, insertion certification, patient management software, and compounding pharmacy network. Revenue-sharing model. Strong brand recognition — many patients ask for "Biote" specifically.

→ Best entry point for practices new to hormone pellets

Evexias Medical Centers / EvexiPEL

PELLET PROGRAM • TRAINING INCLUDED

Pellet training and certification program. Ongoing clinical support. Patient management platform. Compounding through affiliated pharmacies. Competitive alternative to Biote.

→ Good Biote alternative — compare pricing and support before choosing

Lab partners — for in-house or send-out testing

LabCorp / Quest Diagnostics

REFERENCE LAB

Standard reference labs. Widely accepted by insurance. Broad hormone panel availability. Most patients already familiar. Order through your EMR or paper requisition. Results in 24–72 hours typically.

→ Default choice for most practices — familiar to patients, insurance-friendly

ZRT Laboratory

SPECIALTY LAB • SALIVA/BLOOD SPOT

Specialty hormone testing including saliva, dried blood spot, and urine. Particularly useful for monitoring topical hormone absorption (saliva reflects tissue levels better than serum for transdermal). Good for functional medicine approach.

→ Use when serum testing doesn't correlate with symptoms

Dutch Test (Precision Analytical)

SPECIALTY • URINE HORMONE METABOLITES

Dried urine testing for comprehensive hormone panel including metabolites. Shows how the body is processing and breaking down hormones — information serum tests miss. More expensive but clinically valuable for complex cases.

→ Best for complex or refractory hormone cases — not needed for routine monitoring

Telehealth platform partners — for collaborative models

Midi Health / Winona / Alloy

TELEHEALTH PRESCRIBING

Telehealth platforms prescribing menopause and hormone therapy. Option to refer patients who need prescribing but your scope or model doesn't support it. Some allow affiliate/referral arrangements. Patients receive pharmacy-direct medications.

→ Referral option if you're not prescribing yet — keeps patients in your ecosystem

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Pre & Post Consent Forms

What must be in every hormone therapy consent — and the post-initiation acknowledgment

LEGAL NOTE — HAVE YOUR CONSENT FORMS REVIEWED BY A HEALTHCARE ATTORNEY

These templates cover the required clinical elements of hormone therapy informed consent. Before using in your practice, have a licensed healthcare attorney in your state review and approve your consent documents. State-specific requirements vary. This is an educational framework, not legal advice.

PRE-TREATMENT Informed Consent — Hormone Replacement Therapy

Patient reads, initials each section, signs at the bottom. Provider co-signs. Witness signature recommended.

1. Nature of treatment: I understand that I am being offered hormone replacement therapy (HRT/BHRT) to address symptoms of hormonal imbalance or deficiency. I understand this is a medical treatment that requires ongoing monitoring and follow-up.

2. Medications to be used: I have been informed of the specific hormone(s) to be prescribed, the delivery method, dosing, and frequency. These have been explained to me in terms I understand. _____ initials

3. Expected benefits: I understand that potential benefits may include improvement in energy, sleep, mood, libido, hot flashes, vaginal symptoms, mental clarity, and body composition. I understand results vary and are not guaranteed. _____ initials

4. Known risks — estrogen therapy: I have been informed of the following potential risks: increased risk of blood clots (DVT/PE), stroke, breast tenderness, spotting or bleeding, headache, nausea, and a possible increased risk of breast cancer with long-term combined HRT use. I understand the evidence on breast cancer risk is complex and has been discussed with my provider. _____ initials

5. Known risks — testosterone therapy: I have been informed of: acne, oily skin, hair thinning or loss, clitoral/penile enlargement (dose-dependent), mood changes, increased red blood cell production (polycythemia), voice changes (rare in women at physiologic doses), and in men — testicular atrophy, fertility impact, and PSA elevation. _____ initials

6. Compounded medications: If applicable — I understand that compounded hormones are not FDA-approved finished drug products. They are prepared by a licensed compounding pharmacy and are not subject to the same pre-approval process as FDA-approved medications. I accept this and consent to receiving compounded hormones. _____ initials

7. Monitoring requirements: I agree to follow the recommended lab monitoring schedule, including repeat labs at 6–8 weeks after initiation and every 3–6 months thereafter. I understand that my treatment may be adjusted or discontinued based on lab results or symptom response. _____ initials

8. Alternatives: I have been informed that non-hormonal alternatives exist for my symptoms and have been given the opportunity to discuss these. I am choosing hormone therapy after understanding these alternatives. _____ initials

9. Right to withdraw: I understand that I may discontinue hormone therapy at any time. I understand the potential effects of stopping therapy and will discuss discontinuation with my provider. _____ initials

10. Pregnancy and contraception: I confirm I am not pregnant. If I am of childbearing age, I understand that hormone therapy is contraindicated in pregnancy and I will use appropriate contraception. I will notify my provider immediately if I become pregnant. _____ initials

Patient signature: _____ **Date:** _____

Provider signature: _____ **Date:** _____

Witness: _____ **Date:** _____

POST-INITIATION Acknowledgment — 6-Week Follow-Up

Completed at the 6-8 week follow-up visit after labs are reviewed

Lab results reviewed: I confirm that my provider has reviewed my follow-up hormone labs with me and explained the results. I understand my current levels and how they relate to my symptoms. _____ initials

Dose adjustment: My dosing has been: ☐ Continued as prescribed ☐ Increased ☐ Decreased ☐ Changed delivery method. The reason for any change has been explained to me. _____ initials

Side effects experienced: I have reported the following side effects since starting therapy (if none, write "none"): _____ These have been addressed by my provider. _____ initials

Ongoing monitoring agreement: I agree to continue with the recommended monitoring schedule. I understand that ongoing monitoring is required for safe long-term hormone therapy. _____ initials

Patient signature: _____ **Date:** _____

Provider signature: _____ **Date:** _____

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Do's & Don'ts — Hormone Therapy in Practice

The operational rules that protect your patients and your license

DO — ALWAYS

DON'T — NEVER

- ✓ Obtain and review baseline labs before prescribing any hormone
- ✓ Document the clinical indication for every prescription
- ✓ Use shared decision-making — document that alternatives were discussed
- ✓ Schedule follow-up labs at 6–8 weeks after any initiation or dose change
- ✓ Prescribe progestogen with systemic estrogen in patients with a uterus
- ✓ Monitor PSA in all men on testosterone therapy
- ✓ Monitor hematocrit in all testosterone patients — men and women
- ✓ Use PCAB-accredited compounding pharmacies only
- ✓ Document patient symptoms at every visit — not just labs
- ✓ Maintain a collaborative agreement if required by your state
- ✓ Recommend annual mammograms for women on estrogen therapy
- ✓ Keep consent forms current and reviewed by a healthcare attorney

- ✗ Prescribe hormones without a clinical indication and documented labs
- ✗ Dose to a number — dose to symptom resolution within safe lab ranges
- ✗ Use synthetic progestins (medroxyprogesterone) when bioidentical progesterone is appropriate and available
- ✗ Prescribe without a follow-up plan — abandonment is a liability
- ✗ Use unaccredited compounding pharmacies or unverified online pharmacies
- ✗ Prescribe estrogen without addressing progestogen need
- ✗ Market hormone therapy with unsubstantiated cure or anti-aging claims
- ✗ Prescribe outside your scope of practice or state prescribing authority
- ✗ Ignore patient symptoms even if labs are "within range"
- ✗ Miss PSA monitoring in men — malpractice exposure is significant
- ✗ Prescribe to pregnant patients or those trying to conceive without specialist co-management

Incorporating into your aesthetic practice — the smart workflow

- 1 Add hormone symptom screening to your aesthetic intake form**
Ask every patient about energy, sleep, libido, mood, and weight — even if they came in for Botox. This surfaces hormone candidates you'd otherwise miss. "Have you noticed changes in your energy, weight, or mood in the last year?" opens the door.
- 2 Offer a hormone consultation as a standalone appointment**
45–60 minutes. \$150–\$250 consultation fee (may apply toward first month of care). Covers symptom history, lab orders, and treatment plan discussion. Separate from aesthetic appointments — this is a medical visit.
- 3 Build the monthly management visit**
\$150–\$200/month. Labs every 3–6 months at additional cost. This is your recurring revenue engine. A hormone patient who stays with you is worth \$1,800–\$2,400/year before any aesthetic services.
- 4 Cross-sell naturally — not aggressively**
"Your skin quality will actually improve as your hormones optimize — estrogen directly supports collagen production. A lot of my hormone patients see a noticeable difference in their skin texture over the first 3–6 months." That's education, not sales.



Patient FAQs — Hormone Therapy

The questions every hormone patient asks — answered clearly and compliantly

What is the difference between BHRT and traditional HRT?



Does hormone therapy cause breast cancer?



How long does it take to feel a difference?



Do I have to take hormones forever?



Can women take testosterone?



What will my labs cost?



Is compounded hormone therapy safe?



Can hormone therapy help my skin?



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